

AskRigor - sample end-user report

Visual reference for the YouTube API Services compliance review. This demonstrates the report format shown in the AskRigor Custom GPT.

SYNTHETIC
DE-IDENTIFIED

Demonstration only. Every title, channel, count, finding, and outcome in this document is synthetic. It contains no real user prompt, diagnosis, patient data, commenter identity, or comment text. It is not medical advice.

Structured rehabilitation options - example research summary

Example research target: Compare two materially different structured rehabilitation programs in a de-identified adult population, preserving program dose, stage, outcome horizon, harms, durability, and source-access limitations.

Bottom line

- The inspected sources describe two materially different rehabilitation programs. The sample evidence can describe them separately, but does not support an individual recommendation.
- Public YouTube discussion is used to look for implementation details, reported failures, harms, and durability questions. It is not treated as a controlled estimate of benefit.

Approaches compared

Approach	What was actually examined	Example outcome and horizon
Progressive resistance program	Supervised lower-body resistance sessions, three times weekly for 12 weeks, with progression rules	Walking tolerance and function at 12 weeks
Aquatic mobility program	Pool-based mobility and conditioning sessions, twice weekly for 10 weeks	Pain during activity and function at 10 weeks

What the evidence found

The synthetic formal-evidence example reports different short-term outcomes for the two exact programs. This result cannot be generalized to programs with a different dose, disease stage, supervision level, or comparator. No individual treatment recommendation is authorized by this example.

Public discussions checked

Video	Public-data coverage	Directional result	Boundary
Example progressive program walkthrough Synthetic Rehabilitation Channel 2 Jan 2025	YouTube reported 7 public comments. All 7 top-level comments and replies were checked; 2 records contributed structured findings.	One implementation difficulty and one reported short-term improvement were mapped for follow-up.	Public comments are self-selected reports, may be incomplete, and cannot establish efficacy or frequency.
Example aquatic program session Synthetic Mobility Channel 14 Feb 2025	Comments were disabled.	No community finding was inferred.	The absent public discussion is an access boundary, not evidence that no benefits or harms occurred.

Videos actually audited

1. Example progressive program walkthrough - Synthetic Rehabilitation Channel, 2 Jan 2025

- Program: supervised progressive resistance, three times weekly for 12 weeks.
- Population/stage: the exact synthetic study population.
- Outcome/horizon: walking tolerance at 12 weeks.
- Why nonredundant: the video shows progression rules not present in the aquatic program.
- Public discussion: all 7 API-visible records in this synthetic example were checked.
- Boundary: creator and commenter statements are attributed reports, not proof of benefit.

2. Example aquatic program session - Synthetic Mobility Channel, 14 Feb 2025

- Program: pool mobility and low-impact conditioning, twice weekly for 10 weeks.
- Population/stage: the exact synthetic study population.
- Outcome/horizon: pain during activity and function at 10 weeks.
- Why nonredundant: water-supported exercise is materially different from loaded resistance.
- Public discussion: comments were disabled.
- Boundary: no community conclusion was drawn from unavailable comments.

Videos worth watching

Example progressive program walkthrough, at 04:10

Useful for the creator's demonstration of the synthetic progression rule.

Boundary: The demonstration verifies what the creator presented, not whether the program is effective or appropriate for a particular person.

What remains uncertain

- Whether either exact program applies to a different disease stage or baseline.
- Durability beyond the synthetic follow-up period.
- The frequency of benefits, failures, or harms in the wider population.
- Public comments are not representative, and unavailable or moderated material cannot be observed through the API.

Questions for clinician review

- Does the evidence population match the person's actual stage and constraints?
- Are the program dose and supervision feasible and safe?
- Which outcomes and harms should be monitored, and over what period?

How YouTube API Data is presented

The ordinary end-user report shows linked video identity, channel/date, the program or topic examined, API-reported and retrieved coverage counts, structured findings, and plain-language limitations. It does not show a raw provider response, API key, raw commenter identity, or raw comment text.

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